

SENATE RESOLUTION 119

By Lamar

A RESOLUTION to recognize the week of April 11 through April 17, 2025, as Black Maternal Health Week.

WHEREAS, Black Maternal Health Week was founded by Black Mamas Matter Alliance, Inc., to bring national attention to the maternal and reproductive health crisis in the United States and the importance of reducing maternal mortality and morbidity among Black women and birthing persons; and

WHEREAS, according to the Centers for Disease Control and Prevention (CDC), Black women in the United States are two to three times more likely than White women to die from pregnancy-related causes; and

WHEREAS, Black women and people living in low-income and rural communities in the United States are the most likely to suffer from life-threatening pregnancy complications, known as "maternal morbidities"; and

WHEREAS, maternal mortality rates in the United States are among the highest in the developed world, with 23.8 deaths per 100,000 live births in 2020, 32.9 in 2021, 22.3 in 2022, and 18.6 in 2023; and

WHEREAS, the United States has the highest maternal mortality rate among affluent countries, in part because of the disproportionate mortality rate of Black women; and

WHEREAS, according to the 2025 CDC Report, in 2023, the U.S. maternal mortality rate decreased for White (14.5), Hispanic (12.4), and Asian (10.7) women but increased to 50.3 deaths per 100,000 live births for Black women; and

WHEREAS, Black women are fifty percent more likely than all other women to give birth to premature, low birthweight, and very low birthweight infants; and

WHEREAS, the high rates of maternal mortality among Black women span across income levels, education levels, and socioeconomic status; and

WHEREAS, the Centers for Disease Control and Prevention has found that more than eighty percent of pregnancy-related deaths are preventable; and

WHEREAS, the leading causes of maternal mortality among Black women and birthing persons include eclampsia, preeclampsia, postpartum cardiomyopathy, and obstetric embolism, and these conditions impact Black women and birthing people disproportionately; and

WHEREAS, structural racism, gender oppression, and the social determinants of health inequities experienced by Black women in the United States significantly contribute to the disproportionately high rates of maternal mortality and morbidity among Black women; and

WHEREAS, racism and discrimination play a consequential role in maternal healthcare experiences and outcomes of Black birthing people; and

WHEREAS, the overturn of *Roe v. Wade* impacts Black women's and birthing people's right to reproductive health care and bodily autonomy and further perpetuates reproductive oppression as a tool to control women's bodies; and

WHEREAS, a fair and wide distribution of resources and birth options, especially with regard to reproductive healthcare services and maternal health programming, is critical to addressing inequities in maternal health outcomes; and

WHEREAS, states and rural counties with higher Black population rates have severe maternity care deserts, where there are no hospitals or birth centers offering obstetric care nor any obstetric providers, and there is diminished access to reproductive healthcare providers due to low Medicaid reimbursements, rising costs, and persistent healthcare workforce shortages; and

WHEREAS, maternity care deserts lead to higher risks of maternal morbidity and mortality, as most complications occur in the postpartum period when birthing people are far away from their providers; and

WHEREAS, Black midwives, doulas, perinatal health workers, and community-based organizations provide holistic maternal care and support but face structural and legal barriers to licensure, reimbursement, and provision of care; and

WHEREAS, Black women and birthing persons experience increased barriers to accessing prenatal and postpartum care, including maternal mental health care; and

WHEREAS, even as there is growing concern about improving access to mental health services, Black women are least likely to have access to mental health screenings, treatment, and support before, during, and after pregnancy; and

WHEREAS, Black pregnant and postpartum workers are disproportionately denied reasonable accommodations in the workplace, leading to adverse pregnancy outcomes; and

WHEREAS, Black pregnant people disproportionately experience surveillance and punishment, including shackling incarcerated people during labor, drug testing mothers and infants without informed consent, separating mothers from their newborns, and criminalizing pregnancy outcomes such as miscarriage; and

WHEREAS, Black women and birthing people experience pervasive racial injustice in the criminal justice, social, and healthcare systems; and

WHEREAS, justice-informed, culturally congruent models of care are beneficial to Black women; and

WHEREAS, an investment must be made in:

- (1) Maternity care for Black women and birthing persons, including care led by the communities most affected by the maternal health crisis;

- (2) Continuous health insurance coverage to support Black women and birthing persons for the full postpartum period at least one year after giving birth; and
  - (3) Policies that support and promote affordable, comprehensive, and holistic maternal health care that is free from gender and racial discrimination, regardless of incarceration;
- now, therefore,

BE IT RESOLVED BY THE SENATE OF THE ONE HUNDRED FOURTEENTH GENERAL ASSEMBLY OF THE STATE OF TENNESSEE, that we recognize the week of April 11 through April 17, 2025, as Black Maternal Health Week to underscore the importance of reducing maternal mortality and morbidity among Black women.

BE IT FURTHER RESOLVED, that we recognize:

- (1) Black women are experiencing high, disproportionate rates of maternal mortality and morbidity;
- (2) The alarmingly high rates of maternal mortality among Black women are unacceptable and unjust;
- (3) In order to better mitigate the effects of systemic and structural racism, we must work toward ensuring that the Black community has:
  - (A) Safe and affordable housing;
  - (B) Transportation equity;
  - (C) Nutritious food;
  - (D) Clean air and water;
  - (E) Environments free from toxins;
  - (F) Decriminalization, removal of civil penalties, end of surveillance, and end of mandatory reporting within the criminal justice and family regulation system;
  - (G) Safety and freedom from violence;
  - (H) A living wage;

- (I) Equal economic opportunity;
  - (J) A sustained and expansive workforce pipeline for diverse perinatal professionals; and
  - (K) Comprehensive, high-quality, and affordable health care, including access to the full spectrum of reproductive care;
- (4) In order to improve maternal health outcomes, we must fully support and encourage policies grounded in the human rights, reproductive justice, and birth justice frameworks that address maternal health inequities;
- (5) Black women and birthing persons must be active participants in the policy decisions that impact their lives;
- (6) In order to ensure access to safe and respectful maternal health care for Black birthing people, the Black Maternal Health Momnibus Act must be passed, as well as other legislation rooted in human rights that seeks to improve maternal care and outcomes; and
- (7) Black Maternal Health Week is an opportunity to:
- (A) Deepen the national conversation about Black maternal health in the United States;
  - (B) Amplify and invest in community-driven policy, research, and quality care solutions;
  - (C) Center the voices of Black Mamas, women, families, and stakeholders;
  - (D) Provide a national platform for Black-led entities and efforts addressing maternal and mental health, birth equity, and reproductive justice;
  - (E) Enhance community organizing on Black maternal health; and

(F) Support efforts to increase funding and advance policies for Black-led and -centered community-based organizations and perinatal birth workers that provide the full spectrum of reproductive, maternal, and sexual health care.